SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

# Trifluridine/tipiracil (Lonsurf) monotherapy

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Previously treated metastatic colorectal cancer · NCCP 00382 · version 4 · 28-day cycle

| Patient name | DOB | Hospital no. | Date |
|---|---|---|---|
| | | | |

**Treatment context**
This oral chemotherapy aims to control colorectal cancer that has spread after earlier treatments, or when those treatments are unsuitable. It combines two ingredients in one medicine. Your team discusses the likely benefit, uncertainty, alternatives and your priorities.

**Schedule**
Take the prescribed morning and evening tablets on Days 1–5, have no Lonsurf on Days 6–7, then take morning and evening tablets on Days 8–12. Days 13–28 are tablet-free. A new 28-day cycle starts only when the team authorises it.

## 1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

## 2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

**Trifluridine/tipiracil**
ORAL CYTOTOXIC CHEMOTHERAPY

- ☐ Low blood counts: infection, anaemia and bleeding
- ☐ Serious infection or febrile neutropenia
- ☐ Nausea, vomiting, diarrhoea and dehydration
- ☐ Fatigue, reduced appetite and mouth soreness
- ☐ Kidney impairment can increase treatment toxicity
- ☐ Pregnancy harm and important contraception requirements

Discuss the disease-control aim, expected benefit and uncertainty, alternatives including supportive care without this treatment, infection/bleeding risk, bowel symptoms and dehydration, oral dosing responsibilities, kidney/liver suitability, fertility, pregnancy prevention and your right to reconsider.

**Individual aim / benefit, alternatives, material risks and questions:**

## 3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

**Prototype only.** This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00382 v4.

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Developed by Dr Paul O'Brien